

25%

About 25 percent of twin births in the US happen vaginally. But mothers expecting triplets or more are usually encouraged to opt for a cesarean section to minimize risk.

Q Will I have to have my labor induced if I'm carrying multiples?

The majority of multiples arrive prematurely, but if yours don't, your doctor will recommend that you be induced or scheduled for cesarean section before 40 weeks. In all pregnancies there is a risk of stillbirth if the baby remains in utero too long, and for multiples that risk happens earlier. For twins this means early induction (see p.203) at around 37 to 38 weeks. For greater multiples, it will mean an early cesarean section, usually at 34 weeks for triplets, and at 32 weeks for quads or more.

Q My doctor says that there might be quite a throng in my delivery room. Why?

Multiple births are more complicated than singletons, so when you go into labor there may be a team of clinicians for each baby. Teams may be made up of your doctor, an anesthesiologist, nurses, and pediatricians. This means that if you are having twins there will be twice as many people in the room as for a single baby; if you're having triplets there will be three times as many, and so on. The number of resuscitators (the bedlike carts that have oxygen equipment for newborns; see p.86) and other items of equipment in the room will also increase. This can all seem pretty overwhelming, but stay focused on your birth partner and your babies, be reassured that so many people are there to help you, and always feel free to ask what someone's role is if it's not immediately obvious.

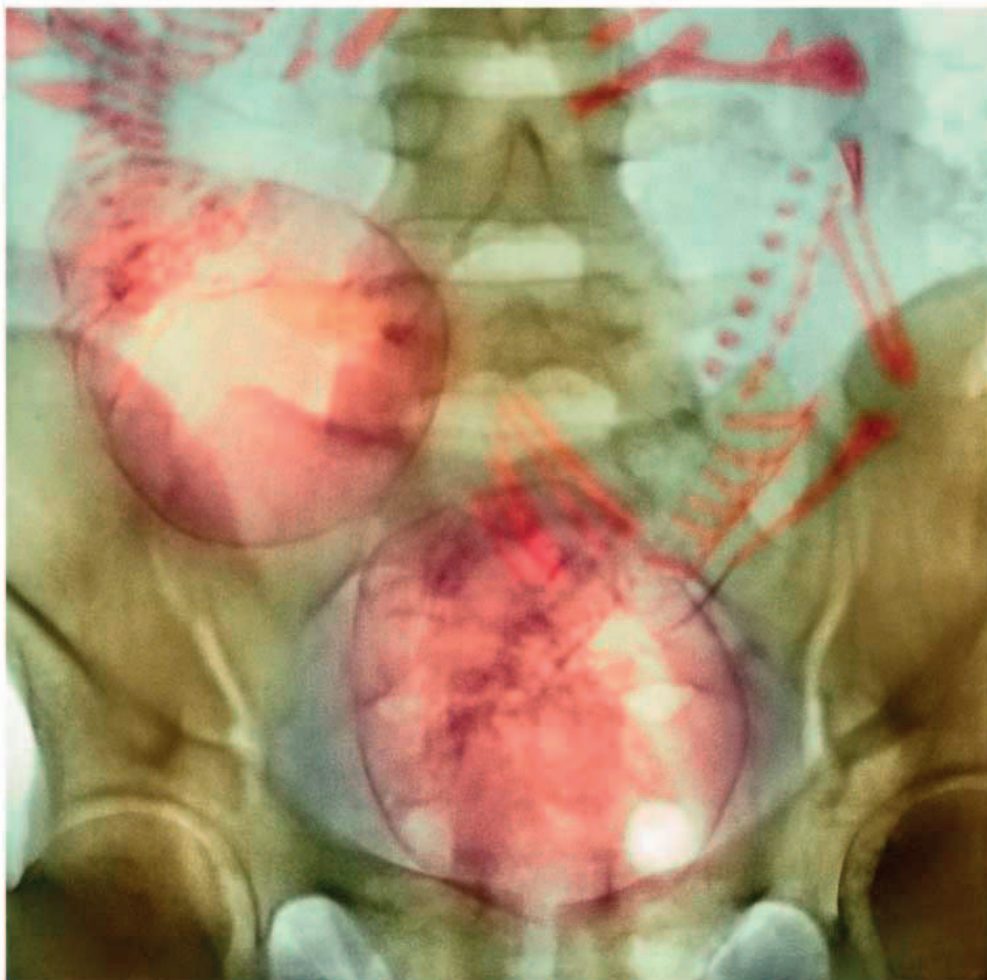
Q Is delivering the placenta(s) and membranes different for twin births?

Usually, after a vaginal twin birth you will be encouraged to have an oxytocin injection in order to speed along the third stage of labor. A managed third stage labor can help stem postpartum bleeding: you are at greater risk of excessive bleeding after a twin birth. The doctor will check that the placenta or placentas are intact, the same as for a single birth.

If your twins are the same sex and have a placenta each and you are not sure whether they are identical or nonidentical, your doctor will examine the placentas and membranes to see if it's possible to figure it out. If it isn't possible, you can opt for private DNA testing if you want to find out definitively. Of course, with many identical twins, identity becomes fairly obvious within a few years.

Q Will my babies go to the neonatal intensive care unit? Will this hamper the bonding process?

If one or all of your babies are very tiny because they are born early, they may need to spend a little time in the neonatal intensive care unit (NICU) after birth to help them thrive and make sure they are continually monitored. Try not to worry. As long as it's medically safe to do so, you'll be able to hold them first. Remember that bonding with your babies began while they were in the uterus, so you already have a good start. Take photographs of each baby and of the babies together so when they aren't with you looking at the photographs will help to trigger some of the bonding hormones that you'd have if you were holding them. It can also help trigger the let-down reflex that enables you to express your own milk for your babies to drink. It will help you adjust to the idea of parenthood, too.



Full-term twins in the uterus Both babies are head down—their skulls point to their mother's pelvis, and their spines are seen curving upward. On the right, the baby's head is engaged in the pelvis, ready for birth.